TYPICAL SIGNS & SYMPTOMS OF PSYCHIATRIC ILLNESS

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Objectives:

At the end of this lecture, each student should be able to:

- Differentiate between psychiatric signs and symptoms.
- Determine the signs and symptoms as presented by the patient.
- Record signs and symptoms in nursing record accurately.

Signs: are clinicians' observations and objective findings

Symptoms: are subjective experiences described by the patient.

A syndrome: is a group of signs and symptoms that occur together as a recognizable condition that may be less than specific than a clear-cut disorder or disease.

Composition of mind

The mind is composed of three spheres that represented the aspects of mental functioning. They are interrelated to each other.

- Intellect: it includes consciousness, memory, orientation, perception, attention and concentration, thought process, insight, knowledge judgment, and intelligence.
- Emotion: it is the inner feeling of experiences of the individual.
- Behavior: it is the conduct of the individual

Component of mind.(disorders of thought –disorder of emotion-disorder of behavior)

Type of disorder:

1-Thought disorder **2-Disturbance in perception.**

3- Unreality states. **4- Disorder of memory.**

5- Orientation , disorientation. **6- Judgment.**

7- Insight. **8- Attention and concentration.**

9- Disorder of consciousness. **10- Disorder of emotion.**

11- Disorder of behavior

I.Disorder of Thinking:

When the order, sequence, attraction and alternations are lacking, thoughts do not fulfill their function, and ideas are just put together in a disconnected and loose manner. This is the basic defect in formal thought disorder.

Ability to thinking:

Concrete thinking:

Literal thinking, limited use of metaphor without understanding of nuances of meaning; one dimensional thought (concretenss versus abstractness)

Ex: اتصدقين ... صرت عالحديدة عالحديدة؟؟؟فين الحديدة ما اشوفه

A. Abstract thinking:

Ability to appreciate nuances of meaning; multidimensional thinking with ability to use metaphors and hypotheses appropriately.

اتصدقين ... صرت عالحديدة ..... ربنا يعوض عليكي

c. Autistic thinking: preoccupation with inner, private world. term used somewhat synonymously with dereism or(self-centered).

A. General disturbances in the stream (rate) of thoughts:

— It is the amount (quantity) and speed of thinking.

—

Disturbance in the stream of speech:

- pressure of thinking
- Blocking of thinking
- Poverty of thinking

1. ***Pressure of speech***

The speech is increased in both quantity and rate and is difficult to interrupt.

— ***Increased rate in the case of :***

The rate of speech may be increased in mania.

— ***Increased quantity in the case of :***

The quantity of speech may be increased in: Mania Anxiety.

2. ***Poverty of speech***

The speech is markedly reduced in quantity, with perhaps only occasional monosyllabic replies to questions.

— ***Decreased rate in the case of :***

The rate of speech may be decreased in: Dementia Depression.

3. **Blocking (or sudden stoppage)**

Occurs in schizophrenia.

Patient breaks or stops discussion or speech before a thought or idea is finished. After a brief pause ; the patient indicates no recall of what was being said or was going to be said.

B. General disturbances in the form of thoughts:

Disorders of the stream of thoughts are not separable from formal thought disorders, they are also related to the association and the goal directed sequences as well as the speed of production, expression and succession of thoughts.

-Neologism: new word created by the patient, often by combining syllabless of other words for idiosyncratic psychological reasons.

Ex : هاكون متاتا

- **Word salad:** incoherent mixture of words and phrases.
- **circumstantiality:** indirect speech that is delayed in reaching the point but eventually gets from original point to desired goal; characterized by an over inclusion of details.
- **Tangentiality:** inability to have goal-directed associations of thought; patient never gets from desired point to desired goal.

Ex : رحت يوم الاحد الصبح عند ابو عبد الله في المكتب
و .....
تعشيت اليوم في مطعم لا يفوتك

- **Incoherence (Loss of association):** thought, that generally is not understandable; running together of thoughts or words with no logical or grammatical connection, resulting in disorganization.

Ex : نمت العشى داخل الباب

- **Perseveration:** persisting response to a prior stimulus after a new stimulus has been presented often associated with cognitive disorders.

وقت الافطار .....,هم ماجوش ليه.. انزل مع زميلك ... هم ماجوش ليه....خد العلاج... هم ماجوش ليه.

- **Verbigeration:** meaningless repetition of specific words or phrases (repeat the one word or phrase over and over again).
- **Echolalia:** psychopathological repeating of words or phrases of one person by another; tends to be repetitive and persistent.

-Flight of ideas (pressure of speech): rapid, continuous verbalizations or plays on words producing constant shifting from one idea to another; the ideas tend to be connected, and in the less severe form a listener may be able to follow them.

--Pressure of speech: rapid speech that is increased in amount and difficult to interrupt.

-Clang association: association of words similar in sound but not in meaning; words have no logical connection may include rhyming and punning.

ex: الجو بديع والدنيا ربيع قفل على كل المواضيع .

- **Irrelevant answer:** Answer that is not in a harmony with te question asked.

Ex : تشربي ايه؟ الكتاب عند امل

- **Blocking:** abrupt interruption in train of thinking before a thought or idea is finished; after a brief pause; the person indicates no recall of what was being said or was going to be said.

Ex: ……..رحت اليوم السوق وشفت فستان لونه

-Poverty of speech: restriction in the amount of speech used; speech that is brief and uncommunicative.

- **Non spontaneous speech:** verbal responses given only when asked or spoken to directly; no self-initiation of speech.
- **Poverty of content of speech:** speech that is adequate in amount but conveys little information because of vagueness, emptiness, or stereotyped phrases.

C. Specific disturbances in the content of thoughts:

It includes:

1-Delusion. **2-Obsession&Compulsion.**

3-Preoccupation. **4-Suicidal ideation.**

5-Phobia.

1-Delusion:

Delusion: it is a false, fixed belief, not based on external reality and not consistent with patient's intelligence, level of education, and cultural background, that cannot be corrected by reasoning or logical argument.

***- Systematized delusions**: false belief or beliefs that appears to be logical and united by single event or theme.

***- Unsystematized delusions:** group of delusions that can not beliefed or related to each other.

A. ***Mood congruent delusion:*** delusion that is parallel or has association with mood.

ex., a depressed patient believes that he or she is responsible for the destruction of the world.

B. ***Mood in congruent delusion****:* delusion with content that has no association to mood.

ex., a depressed patient that has a pleasurable delusion

*DEPRESSIVE DELUSION:

.Nihilistic delusion: false feeling that the self, others, or the world is nonexistent or ending. Or, false belief that a part of his body does not exist or he does not exist (dead).

. Delusion of poverty*:* false belief that one is deprived or will be deprived of all material possessions. Or, the patient believes that he lost everything in his life.

- Delusion of self-accusation, guilt , sin or(self blame):

False feeling of guilt in which the patient believes that he is unfit to live with other people.

- Somatic delusion (Hypochondriacl): false belief involving functioning of one's body. ex., belief that one's brain is rotting or melting.

* Paranoid delusions:

include persecutory delusions and delusions of reference, control and grandeur (distinguished from paranoid ideation, which is suspiciousness of less than delusional proportions) it includes:

- Delusion of persecution:

False belief that one is being harassed, cheated, or persecuted .

- Delusion of grandeur:

Exaggerated conception of one's importance, power, or identity

- Delusion of reference:

False belief that the behavior of others refers to one self; that events, objects, or other people have a particular and unusual significance, usually of a negative nature; derived from idea of reference in which one falsely feels that one is being talked about by others.

Ex: belief that people on T.V or radio are talking to or about the patient.

Delusion of jealousy: conviction that the spouse has some definite relation with someone else.

- **Delusion of infidelity:** false belief derives from pathological jealousy that one's lover is unfaithful.(it is an extreme of the jealousy delusion).

- **Erotic delusion:** false belief that there is a love story between oneself and a famous person.

- **Litigiouse delusion**: patients who have a pathological tendency to take legal action because of imagined mistreatment. Or, patient often write complaints and sends them to the responsible person.

* Delusion of influence (control):

False feeling that one's behaviors, thought, or feeling are being controlled by external forces:

It includes:

-Thought withdrawal: Delusion that one's thoughts are being removed from one's mind by other people or forces.

-Thought insertion: Delusion that thoughts are being implanted in one's mind by other people or forces.

-Thought broadcasting: Delusion that one's thoughts can be heard by others, as thought they were being broadcast into the air.

Physical signs or sensations as abnormal.

2-Obsession& Compulsion:

-Obsession:

Pathological persistence of an irresistible thought or feeling that can not be eliminated from consciousness by logical effort, which is associated with anxiety. (Also termed rumination).

- **Compulsion:**

pathological need to act on an impulse that, if resisted, produces anxiety; repetitive behavior in response to an obsession or performed according to certain rules, with no true end in itself other than to prevent something from occurring in the future.

3-Preoccupation:

Centering of thought content around a particular idea associated with strong affective tone.

4-Suicidal ideation:

It is a recurrent idea affecting the individual to put an end by himself to his life.

5-Phobia: persistent, irrational, exaggerated and in variably pathological dread of some specific type of stimulus or situation, results in a compelling desire to avoid the feared stimulus.

A. ***Zoo phobia (Specific phobia):*** circumscribed dread of a discrete object or situation (for ex., dread of spiders or snakes).

B. ***Social phobia:*** dread of public humiliation, as in fear of public speaking, performing, or eating in public.

C. ***Acrophobia:*** fear of high places.

D. ***Agoraphobia:*** fear of open places.

E. ***Claustrophobia:*** fear of closed places.

F. ***Algophobia:*** fear of pain.

G. ***Erythrophobia:*** fear of red (refers to a fear of blushing).

H. ***Xenophobia:*** fear of strangers.

V. Speech:

Ideas, thoughts, feelings as expressed through language, communication thought the use of words and language.

A. Disturbances in speech:

- **Pressure of speech:** rapid speech that is increased in amount and difficult to interrupt.
- **Poverty of speech:** restriction in the amount of speech used; replies may be monosyllabic.
- **Non spontaneous speech:** verbal responses given only when asked or spoken to directly; no self-initiation of speech.
- **Poverty of content of speech:** speech that is adequate in amount but conveys little information because of vagueness, emptiness, or stereotyped phrases.
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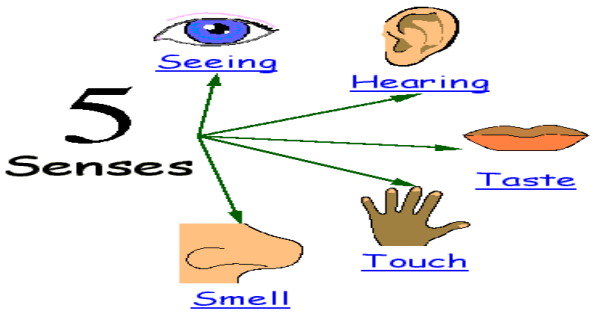
- **Dysarthria** : صعوبة في التعبير**:** difficulty in articulation, not in word finding or in grammar.
- **Stuttering:** frequent repetition or prolongation of a sound or syllable, leading to markedly impaired speech fluency.

التأتأة: تكرار المتكرر أو إطالة صوت أو مقطع، مما يؤدي إلى ضعف ملحوظ طلاقة الكلام.

VI. Perception:

Process of transferring physical stimulation into psychological information; mental process by which sensory stimuli are brought to awareness.

Disturbances of perception:



- **Hallucination:**

False sensory perception **not** associated with real external stimuli; there may or may not be a delusional interpretation of the hallucinatory experience.

a. Auditory hallucination:

False perception of **sound**, usually voices but also other noises, such as music; most common hallucination in psychiatric disorders.

b. Visual hallucination:

False perception involving sight consisting of both formed **image** (ex. people) and unformed images (ex. flashes or light) most common in medically determined disorders.

c. Olfactory hallucination:

False perception of **smell**; most common in medical disorders.

d. Gustatory hallucination:

False perception of taste, such as unpleasant taste caused by an uncinate seizure; most common in medical disorders.

e. Tactile hallucination:

False perception of **touch** or surface sensation, as from an amputated limb (phantom Limb), crawling sensation on or under the skin (formication).

f. Somatic hallucination:

False sensation of things occurring in or to the **body**, most often visceral in origin.

- **Illusion:**

Misperception or misinterpretation of **real external** sensory stimuli.

VII. Memory:

Function by which information stored in the brain is later recalled to consciousness.

A. Disturbances of memory:

- ***1- Amnesia:*** Partial or total inability to recall past experiences; may be organic or emotional in origin.

  a. Anterograde: amnesia for events occurring after a point in time (**loss of memory for recent event.)**.

  b. Retrograde: Amnesia prior to a point in time (**loss of memory for remote event.)**.

  c. **Total amnesia:** loss of memory for recent and remote event.

  d. **Circumscribed amnesia:** loss of memory for limited time.

- ***2- Paramnesia:***
  - **falsification** of memory by distortion of recall(**patient adds fraises details to a true memory).**

- ***Confabulation:*** unconscious filling of gaps in memory by imagined or untrue experiences that patient believes but that have no basis in fact; most often associated with organic pathology.

- ***Déjà vu:*** illusion of visual recognition in which a new situation is incorrectly regarded as a repetition of a previous memory.

- ***jamais vu phenomena:*** in which familiar situation is experienced as novel

3- Hypermnesia: exaggerated degree of retention and recall(**it's excessive memory, the patient mention even unnecessary details).**

B. Levels of memory:

- **Immediate**: reproduction or recall of perceived material within seconds to minutes.
- **Recent**: recall of events over past few days.
- **Recent past**: recall of events over past few months.
- **Remote**: recall of events in distant past.

IX. Insight:

Ability of the patient to understand the true cause and meaning of a situation (such as a set of symptoms).

N.B. A patient with no insight will have poor judgment towards his social , financial and domestic problem

a. Intellectual insight:

Understanding of the objective reality of a set of circumstances without the ability to apply the understanding in any useful way to master the situation.

b. True insight:

Understanding of the objective reality of a situation, coupled with the motivation and the emotional impetus to master the situation.

c. Impaired insight:

Diminished ability to understanding the objective reality of a situation.

X. Judgment:

Ability to assess a situation correctly and act appropriately within that situation.

a. Critical judgment:

Ability to assess, discern, and choose among various options in a situation.

b. Automatic judgment:

Reflex performance of an action.

c. Impaired judgment:

Diminished ability to understand a situation correctly and to act appropriately.

II- Emotion

A complex feeling state with psychic, somatic, and behavioral components that related to **affect** and **mood**.

A. Affect:

Observed expression of emotion; may be inconsistent with patient's **description** of emotion.

- **Appropriate affect:** condition in which the emotional tone is in harmony with the accompanying idea, thought, or speech.
- **Inappropriate affect:** disharmony between the emotional feeling tone and the idea, thought or speech accompanying it.
- **Blunted affect**لهجة الشعور **:** A disturbance in affect manifested by a severe reduction in the intensity of externalized feeling tone.
- **Restricted or constricted affect:** reduction in intensity of feeling tone less severe than blunted affect but clearly reduced.
- **Flat affect:** Absence or **near absence** of any sign of affective expression; voice monotonous, and face immobile.
- **Labile affect:** rapid and abrupt change in emotional feeling tone unrelated to external stimuli.

B. Mood:

A pervasive and sustained emotion **subjectively** experienced and reported by the patient and observed by others examples include depression, elation, anger.

- **Dysphoric mood:** an unpleasant mood.
- **Euthymic mood:** normal range of mood, implying absence of depressed or elevated mood.
- **Irritable mood:** easily annoyed and provoked to anger.
- **Labile mood (swings):** oscillations between euphoria and depression or anxiety.
- **Elevated mood:** a mood more cheerful than usual.
- **Euphoria:** intense elation with feelings of grandeur.
- **Depression:** psychopathological feeling of sadness.
- **Anhedonia:**انعدام التلذذ loss of interest in and withdrawal from all regular and pleasurable activities, often associated with depression.
- **Grief or mourning:** sadness appropriate to real loss.
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C. Other emotions:

- **Fear:** anxiety caused by consciously recognized and **realistic danger**.
- **Anxiety:** feeling of apprehension caused by anticipation of danger, which may be internal or external**.**
- **Agitation:** severe anxiety associated with motor restlessness.
- **Tension:** increased motor and psychological activity that is unpleasant.
- **Panic:** acute, episodic, **intense** attack of anxiety associated with overwhelming feelings of dread and autonomic discharge.
- **Apathy:** dulled emotional tone associated with detachment or indifference.
- **Ambivalence:** coexistence of two opposing impulses toward the same thing in the same person at the same time.
- **Shame:** failure to live up to self-expectations**.**
- **Guilt:** emotion secondary to doing that is perceived as wrong.

D. Physiological disturbances associated with mood:

Signs of somatic (usually autonomic) by dysfunction of the person, most often associated with depression.

- **Anorexia:** loss of or decrease in appetite.
- **Hyperphagia:** increase in appetite and intake of food.
- **Insomnia:** Lack of or diminished ability to sleep.
- **Hypersomnia:** excessive sleeping.
- **Constipation:** inability or difficulty in defecating.

III. Motor behavior

The aspect of the psychic that includes impulses, motivations, whishes, drives, instincts and craving, as expressed by a person's behavior or motor activity.

- **Echopraxia:** pathological imitation of movements of one person by another.
- **Catatonia:** motor anomalies in nonorganic disorders. (as opposed to disturbances of consciousness and motor activity secondary to organic pathology).***Catalepsy:*** general term for an **immobile position** that is constantly maintained.
- ***Waxy flexibility (cerea flexibilities):*** the person can be **molded** into a position that is them maintained; when the examiner moves the person's limb, the limb feels as if it is made of wax.

- **Negativism:** motiveless resistance to all attempts to be moved or to all instructions.
- **Cataplexy:** temporary **loss of muscle** tone and weakness precipitated by a variety of emotional states.
- **Stereotypy:**نمط ثابت/ رتابة repetitive fixed pattern of physical action or speech.
- **Mannerism:** ingrained habitual involuntary movement.
- **Over activity:**

A. ***Psychomotor agitation:*** Excessive motor and cognitive over activity; usually nonproductive and in response to inner tension.

B. ***Tic:*** involuntary, spasmodic motor movement.

C. ***Sleep walking:*** motor activity during sleep.

D. Akathisia: subjective feeling of muscular tension secondary to antipsychotic or other medication which can cause restlessness, pacing, repeated sitting and standing.

E. Ataxia: failure of muscle coordination, irregularity of muscle action.

I- Consciousness

A state of awareness

Apperception: perception modified by one's own emotions and thoughts.

Sensorium: state of cognitive functioning of the special senses.

A. Disturbances of consciousness

Are most often associated with brain pathology.

- **Disorientation:** Disturbance of orientation in **(ppt)** time, place and person.
- **Clouding of consciousness:** Incomplete clear-mindedness with disturbances in perception and attitudes.
- **Stupor:** Lack of reaction to and unawareness of surroundings.
- **Delirium:** Bewilderedحائر, restless, confused, disoriented reaction associated with fear and hallucinations.
- **Coma:** Profound degree of unconsciousness.

B. Disturbances of attention:

Attention is the amount of effort exerted in focusing on certain portion of an experience; ability to sustain a focus on one activity and ability to concentrate.

- **Distractibility:** التشتت Inability to concentrate attention, attention drawn to unimportant or irrelevant external stimuli.
- **Selective inattention:** Blocking out only those things that generate anxiety.
- **Hyper vigilance:** Excessive attention and focus on all internal and external stimuli, usually secondary to delusional or paranoid states.
- **Trance:** Focused attention and altered consciousness usually seen in hypnosis and ecstatic religious experiences.

. General appearance **:**

Certain psychiatric patients (thought less commonly than in popularly thought), display easily observed outward signs of their illness which should arouse the nurse's immediate interest, and which may in some instances at once provide important clues as to the nature of their disorder.

Facial expression :This is chiefly of significance as an indication of the patient's mood, and on some occasions may be more reliable than his own report on the state of his feelings-he may claim to be cheerful and free from worry, but one look at his face may show this to be quite untrue. Feelings of depression, fear, anxiety or hostility are usually clearly shown by the facial cheerful and free from worry, but one look at his face may show this to be quite untrue and contrast strikingly with the blank, vacant gaze, which is seen in some cases of advanced psychiatric illness. Anxiety or fear may also be indicated by beads of perspiration on the forehead and lips in times of stress.

In some mentally retarded patients, notably in Down's syndrome, the facial appearance is highly characteristic.

Posture :Abnormal postures of the body may result from various diseases of the central nervous system or of the bones, muscles or joints. They may however be observed in some psychiatric patients in the absence of physical disease. The grossly depressed patient may slump in a huddled position, sometimes for hours on end if undisturbed. Other patients, notably those suffering from schizophrenic illness, may adopt strange postures which they are capable of maintaining for long periods, this sign being known as ***catatonia***. The patient's limbs may remain for some time in any position in which they are placed, even a highly unusual or

uncomfortable one, and this state is referred to as ***waxy flexibility***. These abnormalities of posture are based on a disturbance in the tone of the muscles.

Mannerisms :Many people show repeated small movements of a habitual kind- they may have unusual ways of smoking a cigarette, certain typical gestures of the hands, a characteristic way of raising their eyebrows, and so on. These are known as ***mannerisms*** and are not in themselves abnormal. In some varieties of psychiatric illness, however, such movements may be extremely peculiar in their form, sometimes giving the patient a quite grotesque appearance; the face may be repeatedly pulled into strange shapes or some part of the body incessantly and violently scratched.

Dress :Though normal people vary widely in their ideas of what constitutes an appropriate style of dress, occasionally one sees patients whose mode of attire is so utterly freakish as to indicate immediately that they are mentally sick. Particularly in the long-stay wards of mental hospitals, there will be met at times patients who choose to wear clothes of an absurdly out of date style or in an eccentric and hideous mixture of colors; one such male patient wore at all times a purple tie about four inches long, which had originally been a book-mark and which he claimed had a special religious significance for him.

Hygiene : Clues to the personality and illness of the patient will occasionally be picked up by noting the amount of time and attention he devotes to the care of his body. If this is unusually great such behavior is said to know evidence of ***narcissism***, that is, excessive love of self, a disturbance of character structure which is not infrequent in some immature personalities. On the other hand, some patients will show a progressive falling off in their standard of hygiene and a lack of concern for normal cleanliness and neatness; this is particularly common in the mental disorders of old age, or in other conditions where there is physical damage of any kind to the brain but may be noted in any advanced psychiatric illness.